BRIGHTWATER HEALTH PLAN

Medical Policy: Total Knee Replacement

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1. Purpose and scope

This policy sets out when Brightwater Health Plan authorises a total knee replacement before it is given, and what a requesting office must show.

It is used by the Intake Coordinator who opens a review, by the UM Nurse Reviewer who decides it, and by Provider Services Representatives answering requesting offices, as the Utilisation Management Policy describes.

Each service has its own medical policy. The five medical policies share one layout and many of the same sentences; they differ in the diagnoses and sites of section 2, in the coverage criteria of section 4, in how section 5 reads them, and in the documents, care and setting of sections 6, 8 and 9. A request is always reviewed under the policy of its own service, and a criterion in one policy never applies to a request for another service.

The policy applies to members of every Brightwater plan.

2. Service covered

This policy covers total knee replacement, service code ORT-TKR, for these diagnoses:

- M17.11 - unilateral primary osteoarthritis, right knee
- M17.12 - unilateral primary osteoarthritis, left knee
- M17.0 - bilateral primary osteoarthritis of knee
- M17.31 - unilateral post-traumatic osteoarthritis, right knee
- M17.32 - unilateral post-traumatic osteoarthritis, left knee

The service is given at a hospital as an inpatient, or at an ambulatory surgery centre.

A request for another service is reviewed under that service's own policy, never under this one.

3. How this policy is applied

When a request for the service arrives, the Intake Coordinator opens its review and the questionnaire is screened against the criteria of section 4, read as the Prior Authorisation Rules set out.

The criteria are read from this policy and from no other document, and the review records each criterion beside the sentence it was read from.

Each criterion is Met, Not met or Not documented on the questionnaire as received. The latest submission of a request is the request; an earlier submission is never screened.

The questionnaire decides every criterion. The clinical documents attached to the request are evidence for the reviewer, read with the evidence guides; a document never fills a blank answer and never replaces one.

The screening recommends one of the decisions open, and a UM Nurse Reviewer who did not open the review decides, as the Utilisation Management Policy describes.

4. Coverage criteria

Every criterion below must be met for a UM Nurse Reviewer to approve the service. Each criterion is stated once, and a request is screened against all of them:

1 **Minimum age.** The member is aged 18 years or over.

2 **Radiographic severity.** A weight-bearing X-ray shows Kellgren-Lawrence grade 3 or higher.

3 **Conservative treatment.** The member has completed at least 12 whole weeks of conservative treatment.

4 **Knee function.** The Oxford Knee Score is 26 or lower.

5 **Body mass index.** The body mass index is below 40.0.

A criterion is never met by a figure in a letter, and never waived except as it says itself.

5. Reading the criteria

Conservative treatment is treatment of the knee without surgery: physiotherapy, supervised or home exercise, weight management, walking aids or a brace, injections into the knee, pain medicines and activity modification. Its length is the total time spent in conservative treatment, counting overlapping courses once, in whole weeks, as the Prior Authorisation Rules set out.

The Kellgren-Lawrence grade is read from a weight-bearing X-ray of the knee to be replaced, from 0 for no osteoarthritis to 4 for severe osteoarthritis with the joint space lost.

The Oxford Knee Score is the member's own score of pain and function in that knee, from 0, the worst, to 48, the best, completed at a recent clinic visit.

The body mass index is worked out from the height and weight on the questionnaire, as the Prior Authorisation Rules set out; an index the requesting office writes is never used.

The minimum age is read from the member's age on the request. A request for a younger member is referred to a Physician Reviewer, who reviews it under the plan's paediatric guidance.

6. Documentation to send

The requesting office should send with the request the report of a weight-bearing X-ray of the knee with its Kellgren-Lawrence grade; the record of conservative treatment for the knee, with when each course began and ended; the Oxford Knee Score from a recent clinic visit; the member's height and weight measured at that visit; and every long-term condition the member has.

7. When a criterion is not met

A request with a criterion that is not met is referred to a Physician Reviewer, and only a Physician Reviewer may deny a request. A UM Nurse Reviewer may also refer a request whose criteria are all met, when the clinical documents raise a concern the criteria do not cover.

The Physician Reviewer may approve a request that does not meet a criterion when the clinical documents show that the service is necessary for the member, and records why in their own words.

The requesting office may ask to discuss a referred request with the Physician Reviewer before a decision, in a peer-to-peer discussion.

A criterion that is not documented is never on its own a reason to refer a request: the missing answer is asked for first, and the request is referred when the answer does not arrive in time or shows a criterion not met, as the Utilisation Management Policy describes.

8. Care before this service

Before a knee replacement the member should have tried physiotherapy and exercise, weight management where it is needed, walking aids and pain medicines; an injection into the knee may help for a time. A member who smokes should be offered support to stop before surgery, and diabetes should be well controlled.

9. Setting and length of stay

An approved knee replacement is authorised at a hospital as an inpatient, or at an ambulatory surgery centre for a single knee when the surgeon judges it safe. An inpatient operation is authorised for the inpatient days the surgeon requests.

The plan's expected stay is shown to the reviewer and never changes the days authorised; when it is more than one day longer than the days requested, a review during the stay is planned, as the Utilisation Management Policy describes.

10. Questions about this policy

The Medical Policy Committee owns this policy, reviews it every twelve months and issues any change as a new issue of this document, replacing every earlier issue.

Questions about this policy from a requesting office go to Provider Services, which answers them from the medical policies, the Utilisation Management Policy, the Prior Authorisation Rules and the evidence guides. Questions from Brightwater staff go to the utilisation management team.